

Clinical Document Intelligence Report

Patient	Margaret Elaine Forsythe	Age	66
Case ID	CASE-FCFD8253	Sources	3

HIGH PRIORITY

Possible acute cardiac concern; Acute myocardial injury pattern; additional concerns include potential ace inhibitor and entresto interaction, potentially unstable heart failure.

Act first: Escalate to the acute cardiac pathway (Immediate / same day)

Combined Summary

Margaret Elaine Forsythe's consolidated record combines 3 documents: Discharge Summary, Lab Report, GP Consultation Note. Conditions documented across the record include type 2 diabetes mellitus, hypertension, ischaemic heart disease. The highest workflow priority is high, driven by acute myocardial injury pattern. Each action remains subject to human confirmation.

Reasoning mode: Transparent clinical rules with source evidence

Overall recommendation: High-priority coordinated review. Escalate for same-day medication safety review and confirm ramipril is stopped via Prescribing clinician / heart failure team; Arrange early heart failure follow-up via Heart failure service; confirm 5 missing information items before case closure; reconcile conflicting information across the record.

Field	Extracted information
Current conditions	type 2 diabetes mellitus; hypertension; ischaemic heart disease; NSTEMI; iron deficiency anaemia; post-MI; heart failure; CKD Stage 3a
Symptoms and signs	ankle oedema; central chest pain; diaphoresis; shortness of breath; dizziness; nausea; bibasal crackles
Important results	Haemoglobin: 10.9 g/dL; eGFR 68; Creatinine: 118 umol/L; eGFR (CKD-EPI): 52 mL/min/1.73m2; Troponin I (hsTnI) @ 02:55: 142 ng/L; Troponin I (hsTnI) @ 05:55: 389 ng/L; BNP (B-type Natriuretic Peptide): 810 pg/mL; BP: 118/72; BNP: 810 pg/mL; O2 sats: 96%; Creatinine 118 umol/L; HbA1c: 9.2%; eGFR 52; Random Glucose: 13.4 mmol/L; Ferritin: 11 ug/L; TSH: 2.4 mU/L; ferritin 11; HbA1c 9.2%
Confirmed allergies	PENICILLIN — anaphylaxis; Iodinated contrast allergy - mild rash
Adverse reactions	Codeine — causes severe nausea

Safety warnings: Acute myocardial injury pattern: Escalate to the acute cardiac pathway; Potential ACE inhibitor and Entresto interaction: Escalate for same-day medication safety review and confirm ramipril is stopped; Potentially unstable heart failure: Arrange early heart failure follow-up; Poor glycaemic control: Route to the diabetes nurse specialist within 7 days; Incorrect allergy severity may be recorded: Verify the original contrast reaction and correct the allergy record

Medication Reconciliation

Medication	Journey	Current	Confidence	Source
Amlodipine 10 mg OD	On admission: Active on admission	Active on admission	High	discharge_summary_unstructured.pdf
Aspirin 75 mg	On admission: Active on admission -> At discharge: Continued -> Current / documented: Continued	Continued	High	discharge_summary_unstructured.pdf; gp_clinical_note.md
Atorvastatin 80 mg nocte	On admission: Active on admission -> At discharge: Continued -> Current / documented: Continued	Continued	High	discharge_summary_unstructured.pdf; gp_clinical_note.md
Bisoprolol 7.5 mg OD	On admission: Not reliably taking before admission -> Post-discharge review: Dose changed	Dose changed	High	discharge_summary_unstructured.pdf; gp_clinical_note.md
Clopidogrel 75 mg	On admission: Active on admission -> At discharge: Continued -> Current / documented: Continued	Continued	High	discharge_summary_unstructured.pdf; gp_clinical_note.md

Medication	Journey	Current	Confidence	Source
enoxaparin 1 mg/kg BD	Current / documented: Active	Active	High	discharge_summary_unstructured.pdf
Ferrous Sulphate 200 mg BD	Current / documented: New	New	High	discharge_summary_unstructured.pdf; gp_clinical_note.md
Furosemide 40 mg OD	Current / documented: Active -> At discharge: New -> Current / documented: New	New	High	discharge_summary_unstructured.pdf; gp_clinical_note.md
Insulin Glargine 28 units nocte	On admission: Active on admission -> Current / documented: Active	Active	High	discharge_summary_unstructured.pdf; gp_clinical_note.md
Metformin 500 mg BD	On admission: Not reliably taking before admission	Not reliably taking before admission	High	discharge_summary_unstructured.pdf
omeprazole 20 mg	On admission: Active on admission -> At discharge: Continued -> Current / documented: Active	Active	High	discharge_summary_unstructured.pdf; gp_clinical_note.md
Ramipril 10 mg OD	On admission: Active on admission -> Current / documented: Stopped -> At discharge: Stopped -> Post-discharge review: Taken in error	Stopped	High	discharge_summary_unstructured.pdf; gp_clinical_note.md
Sacubitril-Valsartan (Entresto) 24/26 mg BD	Current / documented: New -> At discharge: New -> Post-discharge review: New	New	High	discharge_summary_unstructured.pdf; gp_clinical_note.md

Cross-Document Review

Conflict - Contrast allergy severity (High risk)

discharge_summary_unstructured.pdf: Contrast dye | Iodinated contrast allergy

gp_clinical_note.md: Iodinated contrast allergy | rash, mild, resolving spontaneously

Action: Verify the original reaction and correct the allergy record before future contrast imaging.

Medication safety issue - Ramipril after Entresto initiation (High risk)

discharge_summary_unstructured.pdf: ramipril: Stopped

gp_clinical_note.md: ramipril: taken after discharge

Action: Confirm ramipril is stopped and the remaining supply is removed.

Clinical change - Kidney disease stage (Medium risk)

discharge_summary_unstructured.pdf: Baseline: CKD Stage 2

discharge_summary_unstructured.pdf: More severe/current finding: CKD Stage 3a

Action: Use the latest renal function for medication dosing and follow-up.

Record Completeness

6/10 areas complete. Incomplete areas: Medication reconciliation, Allergies and reactions, Important test results, Social or cognitive context. Specific items to confirm: contrast reaction details, medication adherence history, smoking cessation date, formal cognitive assessment, pending vitamin B12 and folate.

Action Queue

Priority	Action	Route to	Due	Source
High	Escalate to the acute cardiac pathway	Acute cardiac team	Immediate / same day	discharge_summary_unstructured.pdf, lab_report_unstructured.txt
High	Escalate for same-day medication safety review and confirm ramipril is stopped	Prescribing clinician / heart failure team	Immediate / same day	gp_clinical_note.md
High	Arrange early heart failure follow-up	Heart failure service	Within 48 hours	discharge_summary_unstructured.pdf, lab_report_unstructured.txt
High	Verify the original contrast reaction and correct the allergy record	Clinical records team / prescribing clinician	Before end of week	gp_clinical_note.md
High	Route to the diabetes nurse specialist within 7 days	Diabetes nurse specialist	Within 7 days	discharge_summary_unstructured.pdf
Medium	Confirm contrast reaction details	Document review queue	Before case closure	discharge_summary_unstructured.pdf
Medium	Confirm formal cognitive assessment	Document review queue	Before case closure	discharge_summary_unstructured.pdf
Medium	Confirm medication adherence history	Document review queue	Before case closure	discharge_summary_unstructured.pdf
Medium	Confirm pending vitamin B12 and folate	Document review queue	Before case closure	gp_clinical_note.md
Medium	Confirm smoking cessation date	Document review queue	Before case closure	discharge_summary_unstructured.pdf

Record Timeline

1. lab_report_unstructured.txt - 2024-06-10

Type: Lab Report

Conditions: CKD, NSTEMI, heart failure

Evidence: Haemoglobin: 10.9 g/dL, Creatinine: 118 umol/L, eGFR (CKD-EPI): 52 mL/min/1.73m², Troponin I (hsTnI) @ 02:55: 142 ng/L

Next action: Escalate to the acute cardiac pathway Also: Arrange early heart failure follow-up.

2. discharge_summary_unstructured.pdf - 2024-06-14

Type: Discharge Summary

Conditions: type 2 diabetes mellitus, hypertension, ischaemic heart disease

Evidence: eGFR 68, BNP: 810 pg/mL, Creatinine 118 umol/L, eGFR 52

Next action: Escalate to the acute cardiac pathway Also: Arrange early heart failure follow-up Also: Route to the diabetes nurse specialist within 7 days.

3. gp_clinical_note.md - 2024-06-17

Type: GP Consultation Note

Conditions: CKD, post-MI, HF

Evidence: BP: 118/72, O₂ sats: 96%, breathless, Ankles still slightly puffy

Next action: Escalate for same-day medication safety review and confirm ramipril is stopped Also: Verify the original contrast reaction and correct the allergy record.